

**I:** Let's start with familiarity with AI. How familiar are you with AI technologies in general?

**P:** I know the basic ones, the LLMs, ChatGPT, Claude, Google AI overviews. And recently at work we've also started using Medendo, which is a transcription tool that turns sessions into reports, also supported by AI.

**I:** Your organization has already introduced some AI tools as well.

**P:** Yeah, it's very much in the beginning stages, still a pilot. It hasn't gone throughout the entire organization.

**I:** And apart from Medendo, you're not using any AI during work?

**P:** No.

**I:** On a scale of one to ten, how familiar would you say you are with AI?

**P:** Seven.

**I:** Did you disclose to your clients that you're using Medendo?

**P:** Yeah.

**I:** Then the next question is about diagnostics and assessment use. What are your thoughts on AI being used in diagnostic or assessment processes?

**P:** When we started using Medendo this was a really big question, what kind of role is it going to have in the process? The main conclusion we all reached, and something I strongly agree with, is that Medendo makes sense for writing down my thoughts and annotations and creating a report from that. But a person should still be behind actually writing the conclusion. It's just turning notes into a full report, that's where AI could be used. But in terms of assessment, I don't think the technology is even there yet to make an accurate assessment.

**I:** How does diagnostics and assessment work in your day to day?

**P:** If somebody comes in for an autism assessment, they first have an intake of about one and a half to two hours where we ask background questions. Then we start the autism assessment, which is usually up to four hours, where we do the autism DSM-5 interview and score whether they adhere to the criteria. After that we have additional interviews, what we call a developmental anamnesis and a heteroanamnesis, where we talk to someone who knows them well in the present and someone who knew them well in the past, like a parent. We combine all of that and create one report that decides whether the person has autism or something else. What Medendo does is transcribe those interviews and create a report from the transcript, but it's not making any conclusions, not deciding how severely someone is struggling, not saying anything about the nature of the complaint itself. It's just turning notes or transcripts into full sentences.

**I:** So it transcribes the process and writes it into a report, but doesn't draw any conclusions. Is the diagnosing itself an objective process, or is there subjectivity to it as well?

**P:** It's funny you mention that, because it's something we've been checking with the Medendo reports. In the beginning it was more suggestible, actually leaning towards certain conclusions. For example, if somebody was describing depressive symptoms, Medendo would sometimes almost state that we're talking about a depression, without taking into account that the pattern of symptoms might point to something else, like an autistic burnout. Medendo wouldn't consider that and the reports were quite suggestible. They're now looking at how to minimize that. And whenever I get these reports, I never just copy paste them. I go through and delete and edit a lot, especially sentences I find too suggestible or lacking nuance. I think the bias is always going to be there, so you should always edit and check it afterwards, because it's always going to be a bit suggestive.

**I:** Have you considered using AI for preliminary screening?

**P:** We kind of have questionnaires now and those are also good screening tools. I wouldn't necessarily know what the additional benefit would be of replacing that with AI, because the questionnaires already do a really good job of screening, like are there a lot of trauma symptoms, is there really low mood, are we seeing somebody on the brink of a psychosis? There are some really good questionnaires and instrumental tools for that right now.

**I:** And summarizing patient information, that's kind of what you're already doing with Medendo. Do you only use it during intakes or also in general sessions?

**P:** We can use it in treatment as well, then it also writes a little report of what you did in the session. But I've done it and it's fine, I never really use those reports, because the things I find more important or would want to write down, the report doesn't really find those things important. So I usually just make my own, and it doesn't take too much time to write my own report after treatment sessions. But for assessments they're quite handy.

**I:** Why is it more handy in assessments?

**P:** Because when you write an assessment report, a large part of it is already transcribing factual information. Treatment is a little bit more about how did the client really experience this, how were they in the room, what did they notice when they were there, how did they react to the information I was telling them, or how was it for them to tell something? You notice so much more that I think a transcription is not going to catch.

**I:** So in the intake or screening part there's more objectivity, just measurements of actual facts like how many days has this person struggled with sleep. And in normal sessions those things come up as well, but those are not necessarily the parts you find important for the report.

**P:** Yeah, maybe so.

**I:** Then the next question is about monitoring and progress tracking. How do you feel about the role of AI in tracking patient progress over time?

**P:** You know, I was thinking about what you mentioned earlier about AI being used as a screening tool, and I want to come back to that for a second. Right now everybody gets five standard questionnaires and they all have to fill them in, and a person who has a regular mood also has to do the mood questionnaire. I feel like if you have an AI that can, based on the answers they're giving, choose which tools to focus on, like okay this is a client where we should focus on psychosis or personality, as opposed to this one which is more like depression or anxiety, then I can imagine AI being a good tool for deciding which screening tools to even use, based on the answers they're giving or based on an intake questionnaire they've filled out.

**I:** Because now you're basically giving everyone everything, and maybe someone with anxiety has to fill out some depression questionnaire that's a bit unrelated. There might be an opportunity for a pre-screening where you only get screened for things that are relevant to you.

**P:** Yeah, it's not bad if somebody has a regular mood and we still screen on mood, because it's also good to know something is not there. But it could be more effective if somebody screens really high on trauma, that apart from the standard questionnaires the AI could already pinpoint that there is this level of traumatic symptoms and we should also do these additional ones. That's something we don't necessarily do in preliminary screening, or we do it later in the intake. But if you're thinking purely about how to get the most information in the quickest amount of time, then an AI tool in that regard would make sense. But then thinking about monitoring, I'm curious how would AI have access to the progress? That could be through transcripts that the AI is tracking to see if something is getting better or worse, or clients do a questionnaire that they continuously track, but then it wouldn't really be AI because any other data tool could do that. The transcript makes sense, but I'm not sure I can see the added benefit of AI tracking progress, because I also feel like the psychologist is there to track it.

**I:** What are you currently doing for monitoring and tracking?

**P:** A huge part of it, it sounds silly, but you just do an evaluation appointment. You do around ten sessions and then you evaluate with the client. You have your own observations and the client themselves can report how they experienced it, if things got better or worse. In addition to that you also have end of session questionnaires to see, compared to the baseline from a couple of months ago, what changed and what didn't. Based on those numbers you yourself can make a conclusion.

**I:** So after a certain number of sessions, whether that's ten for CBT or more for something like schema therapy, you can do it at the end, in the middle, or at the beginning, you can choose.

**P:** It's not necessarily like you have to evaluate after a set amount of sessions, you just do it naturally after around ten sessions. But maybe you also say, before the client goes on vacation we're doing an evaluation, you kind of pick your own time.

**I:** What concerns, if any, would you have about AI tracking patient progress?

**P:** I have so many concerns. I don't like the idea of AI being really used in this field, and not because I think they're taking over my job, but mostly because the information is so sensitive and we're already doing so much to make sure that people don't even have access, let alone that it gets on a data server. A lot of the big AI companies are US companies, and the rules that are there, or the lack of rules or limitations, it's just really dangerous to know that some client that comes into this Dutch healthcare system could technically have their data accessible to the US government. That's everything that everybody has been dealing with the last couple of years in healthcare and privacy. It would just all go out the window. And then apart from that, psychology is not really a hard science, even though psychology really wants to be a hard science. So much happens in the room and observationally. And I'm also wondering, is the AI we have at the point where it can track those things? I know there's a lot of research happening and people are putting a lot of money into AI, but I'm not really confident that these are the things the big investment companies are interested in developing further before they want to implement it on a wider scale.

**I:** Do you know anything about Medendo's data privacy?

**P:** We asked about it a lot. We as psychologists, we had a couple of meetings where we met the team from Medendo and could ask a lot of questions. People were also curious about where their server is, and they are a Dutch company and it's all on a Dutch server. The reason they were chosen is because they were very upfront with where their data was stored and how they were dealing with it. But they did say that very originally, if you go back far enough, it all still goes back to a US company or a US site. So yeah, it's safe, but also how safe is it?

**I:** Then the next question is about administrative and practical support. How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

**P:** I don't have any problems with that. That sounds fine.

**I:** Would you trust AI-generated reports or would you need to review everything carefully?

**P:** The second part, obviously.

**I:** How much time could this save you?

**P:** Medendo shaves off hours. It can take four or five hours to really write the report, and now it's maybe an hour to an hour and a half to read it all back, change a lot of stuff, write and add things, edit, read it again. I think it takes off around three hours.

**I:** And this is mostly in the intakes, because you said you don't really use it during normal sessions.

**P:** Yeah, this is for the diagnostic process, the assessment part. For treatment I personally don't use it, but I know others do.

**I:** Do you feel like using it during normal sessions would also benefit time-wise?

**P:** Yeah, obviously if it's already written down that takes away the time. I think it's a very personal thing. There are some people who are very meticulous about creating reports and really want it to be their own and will change a lot of stuff, and then you'll end up spending 30 minutes on the report even with Medendo, because you're changing so much. There are other people who are more content with the reports that Medendo is writing, and it would save them a lot of time.

**I:** What concerns would you have about accuracy or liability?

**P:** That's a big one, especially in terms of healthcare and AI. But I also think that the AI that's been implemented in healthcare is very cautious about that as well. We're all quite alert when we look at Medendo reports to see how accurate it is and whether it's being too suggestive. I feel like they're not going to quickly implement AI that's going to decide specific results or do diagnostics already. I would say it's different in healthcare, and mental health care is maybe something else again. It's really not a hard science, it's so suggestive.

**I:** And how about liability?

**P:** Obviously I think that would be a big concern. But if you're the psychologist coupled with a patient and you write a report, whatever you do, you're still the person implementing AI, and I absolutely think it's still your responsibility to check what the AI is doing, whether you stand behind it and whether you think it's accurate. If you are not checking it and it's writing things down that are inaccurate or wrong, it's still your responsibility towards the client.

**I:** Has it changed how you document your work?

**P:** Not really. I think everybody's just adjusting to the AI doing the work they were already doing in terms of writing the report. But I'm not letting Medendo decide how I'm going to do my reports.

**I:** Then the next question is about psychoeducation and between-session tools. What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

**P:** I'm thinking about the added benefit. Sure you could implement it, but what would it add? We already have a lot of online modules that people can do between sessions, and I can already tell patients hate that, they don't want to do it, it's a chore. But then I'm thinking more about the ChatGPT kind of thing, where it feels like there's a person listening and talking back to you, and I've heard patients already talk about that, they do enjoy it and they use it. If you have to do a homework assignment and you can do it with a ChatGPT-style chatbot, I think patients would enjoy that more. If it could give some kind of motivation or reflection, or help if they're stuck on a homework assignment, that would be useful. We always tell patients they can contact us if they're stuck, but there's a level where they don't usually easily do that, maybe they feel like a burden. I feel like if there was a chatbot they would do it more often.

**I:** What if the AI provides information that contradicts your treatment approach?

**P:** Obviously I'm not going to listen to it. Well, it's also not that isolated, because if I work on something I also have a supervisory psychologist who's always seeing the patient with me, so I never see a patient on my own. I think it matters more what that supervisory colleague or my team would say than what the AI would say.

**I:** Would you worry about clients relying too much on AI between sessions?

**P:** I feel like this is a pretty big question. There are certain patients of mine that I know talk a lot with ChatGPT about their problems. I have not yet encountered it becoming problematic or having an influence on our sessions. So in my own experience, I've not seen it go wrong. But what does happen with chatbots is that they give a really specific kind of reflection. Sometimes people have problems with other people in their lives, and it's really helpful to have another person in the room with you, like a psychologist, because the psychologist can also feel what this person is doing in contact with others that's causing these problems. That can be a helpful tool to understand them better or to see where their problems are coming from, and this is something a chatbot doesn't have access to yet. What I often hear is that sometimes patients will be in a specific mood or have specific fears, and chatbots can seem to sympathize with them but can't always give an accurate reflection. They kind of go along with whatever the person is saying, but can't pinpoint that maybe they're wrong about something or not seeing it correctly. And if you ask ChatGPT to contradict everything you say it will do that, but I don't think patients do that when talking about personal problems. The person seeing another person can tell what's happening in ways that aren't always in the verbal information.

**I:** There is some way that chatbots respond where it kind of feels like confirmation bias, like the chatbot just says you're right, or this is why everything is happening. And sometimes it seems to lack an ability to reflect critically on someone, where a psychologist might say, maybe you have a part in this too. And AI just doesn't do that as readily.

**P:** Yeah, and it's also very specifically that ChatGPT maybe is not instructed or programmed to do that often or very quickly. Maybe another chatbot would. And this connects to the confirmation bias point. I have one patient who has told their chatbot to be harsh and to contradict them and not agree with them, and I can tell she is very intelligent and she's also doing really well. But there are other patients that aren't really doing well, and they would never give that instruction to their chatbot. So this vulnerable group of people is also not going to ask their chatbot to be harsh. That instruction, I think, they wouldn't give it.

**I:** Especially for people in the beginning of their progress, if you're depressed and you need to change your thoughts, you don't have the idea to say, try and make me think differently.

**I:** How would you monitor what the AI is telling your clients?

**P:** I think, now also with chatbots you have the entire transcript, so I think if that was just accessible...

**I:** This is about risks and limitations. What risks or concerns do you have about using AI in clinical work?

**P:** The risks, obviously there's the data privacy risk that I think is really big. And the second part would be more what I talked about before, where I think there is a very human element in therapy sessions. It's not something that we need like food and water and sleep. It's something that we've added on in a very, this is a way that we are dealing with problems in this really Western world, that we do therapy sessions. But a big way that therapy has developed, the idea of therapy and sessions and the way we do it, it's very much with the idea in mind that there's a person in the room with you who's listening to you, the human component of it all. And I think that verbally a chatbot or AI could not even mimic it, but could take over that place. And I think the verbal information and giving somebody a space where they feel like they can talk about their problems, or they have a feeling that they're less alone, I do think that AI can have a role in that. You can already see that people do it, there's a whole thing around AI therapy and AI companions and all that stuff. So obviously it is mimicking a certain human element, but I don't think it can mimic it to the extent that allows for that part of, there's a person in the room and the person can hold up a mirror to you or could tell you things because it's a person that's experiencing you. That you can experience somebody being negative and you can reflect that back to them in a way that somebody can't do if you're not really experiencing the other person. And I think that's the human element that an AI chatbot, with the technology we now have, can't do. And I do think you need it in order to have full progress, and for an effective therapy session, for effective therapy in general. But also maybe in five years I'm wrong and the AI can do it. And so what's the point anyway?

**I:** So you talked about data privacy and data security. Have you thought about issues like bias in AI systems? We shortly mentioned it.

**P:** I feel like obviously there is going to be bias in the AI. But maybe the reason I'm less concerned about that is because I'm already very distrustful of everything the AI says. So I'm obviously checking everything it's saying before we implement it or put it in somebody's file. So the way we're using AI right now, it hasn't taken over stuff to the point that we don't have our eyes on it. Everything it's doing, we're still very much reviewing it. So if there is bias, we will take it out.

**I:** What concerns do you have about transparency or explainability of AI decisions?

**P:** So in terms of how much the clients or patients know?

**I:** No, not really. It's more about sometimes AI systems come up with something, but then you don't really know what it's based on or where it came from. So if they come up with something that we should have access to, like the theory they have access to or a research paper that they're pulling something from.

**P:** Oh, that's interesting. I've really never thought about that. I'm thinking about when AI would come up with something on their own in terms of healthcare stuff. If there are moments where we have AI that we're actively using in a creative manner and they're coming up with new things, I feel like obviously we should be able to track where they're getting the information from.

I think with ChatGPT you can also click on certain things and they can give you their sources. If that would be similar to the way the AI is doing things, I think that would be a good way.

**I:** Any other limitations that you want to mention?

**P:** I don't know a lot about AI. I don't know a lot about what is possible there. All I know is that if there is a limitation I have in my head, I'm so sure that people will find a way to make that work in terms of AI. It's not necessarily like the model itself. I don't think the AI itself is bad or evil or should not be used or could not be effective. I think in the model itself or in the technology itself, it can be very effective and can be really useful. I just really think everything outside of it, like how people are implementing it, the people behind what decisions are being made, and what the investments are that are being made, that's where more of my concerns, my fears lie. Because I just don't trust that, or have very little faith in that.

**I:** This is about the client perspective and therapeutic alliance. How do you think clients respond to AI being used in their treatment?

**P:** It really differs a lot. Some people are very open to it because they don't understand it. Other people are open to it because they're big technology nerds and they really like it. And there are maybe, let's say, 50% who don't want it, they don't like it, and they're quite distrustful of it. And it's mostly also because they're very wary of where the information is going to.

**I:** So can you also score this? How do you think clients respond to AI being used in treatment, with a score of one being they don't want it to be used and ten being they want it to be used?

**P:** Four. I'm gonna say three, they really don't like it.

**I:** How might this affect the therapeutic alliance if we were to use AI?

**P:** I do think it's a breach of the relationship you have with your client if you're using something that they're quite wary towards. Do i score this?

**I:** No, this is the open question again.

**P:** I do think it will influence it. Because obviously, it's already a big step for people to come in a room and meet somebody they've never met. And if they're also using a program that they don't like, you obviously become this distrustful figure. So I would say it impacts it negatively.

**I:** Do you think clients will be more or less engaged with AI supported treatment?

**P:** I think it differs, because I know that when we talk about Medendo, for example, and how we're going to use it, people have a lot of questions. They want to know where it's going and they don't really like it. That's usually the main response that we get. But I also know that these are sometimes the same people that do talk to ChatGPT and use it. So it does also seem to be like this question of AI is kind of new and people are just hearing about it sometimes for the first



time. They don't really know what it is or how it's being used. There's a lot of confusion around it. So I think sometimes... What was the question again?

**I:** Do you think clients would be more or less engaged with AI supported treatment?

**P:** I feel like if we did a chatbot between the sessions, they'd love it. But then they're really weird with Medendo again. So I think it's more in the confusion, if they understand what the AI is really doing, then they'd be more open to it.

**I:** Professional identity, autonomy and future conditions. Which aspects of your work should remain strictly human? And what would you need to feel confident about using AI safely in practice?

**P:** Well, all of it. What should stay strictly human? I mean...

**I:** You already mentioned a lot of things, but you can summarize a bit.

**P:** Obviously I think the sessions and the human components, stuff like that. But I'm doubting myself because I'm thinking, how different is it what I'm doing versus what AI is doing, and stuff like BetterHelp? They're saying it's a person behind BetterHelp, but what if it's an AI model? And then it's like, okay, why should it then strictly remain human? I don't know. I feel personally like I think it's important to have a human person in the room who you're doing the sessions with. I think that's a benefit that AI does not have.

**I:** So why do you think these aspects need to stay human?

**P:** I think it's more accurate when it's a person compared to the technology we now have when it's AI.

**I:** And what would you need to feel confident about using AI safely in practice?

**P:** The law needs to be updated. It would make me feel more safe if I knew there were actual laws put in place that were being kept up with the development of AI. That would make me feel more safe.

**I:** Okay, anything more to add?

**P:** Are you making an AI model? What are you doing?

**I:** I'm not making an AI model.

**P:** I have nothing to add. I think this is an interesting time and place where we are trying a lot and we don't really know. I think the question that needs to be asked is the added benefit. What is constantly going to be the added benefit of using AI? And also not the added benefit in terms of, okay, how are we going to make more money out of it, because it's maybe attracting a lot of clients. But does this in the long run mean that clients are helped more? Does it make their complaints go away faster? Does it help with the suffering that they have? Those are, I think,

important things to keep in mind when we talk about the added benefit of AI. Because you could take every person away and just do AI. That's cheaper. That's so much cheaper. But yeah. That's my closing sentence.